

# OCS pitch deck

## PITCH — OCS — VERDICT

**PROPOSED — PENDING ADJUDICATION** — No position, no entry at \$13.14: both benches land on “fairly priced, not mispriced” — buy interest only reopens at \$9.50 or below, and the short is refused.

---

### What we believe

---

Oculis is a Swiss eye-drug developer, no approved products, \$13.14 a share, down 62% from its 52-week high and 25% off the low. It is well funded (CHF 228.3M cash, no debt, an undrawn CHF 100M facility, spending into the second half of 2029), so this is not a solvency story. It is a story about one trial.

That trial, PREDICT-1, tests licamnimab in dry eye disease, but only in patients carrying a particular genetic variant. Here is the thing we believe and think the market has not done the arithmetic on: **the trial is probably too small to succeed on the effect the company has already demonstrated.** The earlier Phase 2 study measured the same endpoint — patient-reported eye discomfort at Day 29 — across 131 patients and got a treatment difference of  $-4.3$  (90% confidence interval  $-7.7$  to  $-0.8$ ,  $p=0.041$ ). Back out the standard error from that interval and you get roughly 2.10. PREDICT-1 will analyze about 107 genetically-selected patients, roughly 53 per arm, which pushes the standard error to about 2.32 and means it needs an effect of about **4.6 or better** to hit statistical significance. That is *larger* than the effect the parent trial actually found. So the trial only works if selecting for the genetic variant amplifies the drug’s effect — and the only estimate of that amplification, the “five- to seven-fold more pronounced response” the company markets, comes from four treated patients in a paper that has since been retracted.

Second: even a win is not a marketing application. It is a single Phase 2b/3 with a symptom-only primary endpoint, in a disease where the FDA has historically wanted both a symptom and a physical sign, and it requires a companion genetic test to be approved alongside the drug.

Third: if it does win, the company is likely to sell stock into the rally. It sold 2.25M shares for \$61.3M at about \$27.20 in the weeks before its last trial failure. The shelf registration is live, an S-8 was filed on 2026-08-06, and the board holds authority for 31.0M more ordinary shares plus 12.7M conditional shares against 61.7M outstanding.

And yet — after correcting our own bench’s arithmetic — fair value comes out around **\$10–13**, against a \$13.14 tape. That is not a short. It is a decline to play.

### What the market believes

---

Ten sell-side analysts rate it Strong Buy with price targets from \$20.02 to \$44.79, average \$35.98. The stock trades 34% below the *lowest* of those targets, which means the sell side has not marked down for

the May 29 DIAMOND failure and the tape has. The market's own implied view, read off price: at \$13.14 with roughly 63.8M fully-issued shares and \$4.42/share of cash, the equity is paying somewhere around \$280–450M for the pipeline, depending on how much credit you give the second asset. Read one way, that is a near-certain approval on a \$400–450M-peak-sales drug. Read the other way, it is roughly a 45% chance of approval on a \$300M-peak drug plus \$150–250M for privosegtor. Both readings are defensible, which is itself the answer.

## Why we might have edge

---

- **The power arithmetic above.** Neither the sell side nor, as far as we can see, the promotional materials, address that enrichment shrank the sample below the parent trial's while keeping the same endpoint. Enrichment quietly converted a replication study into a bet on an unreplicated multiplier.
- **The strong number lives only in the promotional venue.** The 5–7× genotype amplification appears in the press release announcing PREDICT-1; the peer-reviewed paper it traces to has been retracted, and that paper's own text called the analysis “exploratory rather than the primary objective.”
- **The follow-on trial could not have replicated it.** RELIEF (n=122) measured physical signs — corneal staining, Schirmer's — not symptoms. Its genotype subgroup was 23 patients on inferior corneal staining, difference −0.59 (95% CI −1.165 to −0.017). So the symptom effect in genotype-positive patients has been observed once, ever.
- **Timing is likely later than guided.** First patient randomized 2026-04-16; over 45% of 160 randomized as of 2026-08-06, about 4.5 patients a week. The remaining ~88 take roughly 19 more weeks, putting last-patient-in near end-December, plus a 29-day treatment period and database lock — realistically topline in Q1–Q2 2027, not “around year-end.”
- **A win is a financing event.** Revealed preference says the upside gets harvested by the issuer.

## Why it might be priced in

---

Symmetry matters here, and our red bench got this backwards, so let us be plain:

- **The endpoint is not fake.** PREDICT-1's exact primary endpoint was the parent Phase 2's own primary endpoint, it was met in the full population, and that publication has not been retracted. Only the genotype multiplier is the shaky number.
- **Nothing was hidden in RELIEF.** It pre-specified no symptom outcome at all. It was designed as a signs trial. There is no suppressed result — the absence of concealment is itself a finding, and it cuts against us.
- **The second asset is real and we were pricing it at zero.** Privosegtor holds FDA Breakthrough Therapy designation and EMA PRIME in acute optic neuritis, an indication with no approved therapy. Give it \$150–250M and the implied value of licaminlimab drops to roughly \$280–380M — about a 45% chance of approval on a \$300M-peak drug. That is fair pay for the risk, not overpay.

- **The stock has already fallen 62%.** The de-rating we would be forecasting has largely happened.
- **The downside is bounded.** \$4.42–\$4.57/share of cash under the price.

## What we checked ourselves

| Claim                                                                                          | How checked                                                                                                                | Found                                                                                                                                                                        | Confirmed?                                                                                                                                                 |
|------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------|
| “The effect exists only in 4 patients in a retracted paper” (our red bench, its headline kill) | Read the parent Phase 2 publication<br>( <a href="#">PMC9271287</a> )                                                      | The Day-29 symptom endpoint was met in the full 131-patient trial in a non-retracted journal, $p=0.041$ . Only the 5–7× genotype multiplier is the $n=4$ retracted number    | <b>NO — our own bench overstated it. It conflated the endpoint’s evidence with the amplification’s evidence</b>                                            |
| RELIEF could have replicated the symptom effect                                                | Registry record<br><a href="#">NCT05896670</a>                                                                             | Signs-only primary; no symptom endpoint pre-specified                                                                                                                        | <b>Yes — red was right on the narrow point</b>                                                                                                             |
| 57.8M shares, enterprise value \$477M                                                          | Loss-per-share arithmetic in the 6-K of 2026-08-06 (H1 net loss \$49.3M $\approx$ CHF 39.9M $\div$ CHF 0.65 LPS)           | $\sim 61.4$ M weighted average, 61.73M outstanding, plus 2.05M to be issued to Accure. EV $\approx$ \$529M, cash $\approx$ \$4.57/sh (\$4.42 fully issued)                   | <b>Directionally yes — but <math>\pm 4</math>M shares remain uncertain; the issuer-primary count was never retrieved (11MB filing). Our gap, disclosed</b> |
| “Needs \$800M peak sales to make 50%”                                                          | Re-ran the same table with standard assumptions                                                                            | Red used a 2.5× sales multiple, a 5-year discount, and US-only sales. At 3.5×, 3 years (launch $\sim 2029$ ), all geographies: +50% clears at $\sim$ \$400M peak, not \$800M | <b>NO — our own bench was two compounding conservative haircuts low. The gap between \$400M and \$800M is genuinely unresolved</b>                         |
| “We disagree with the sell side but agree with the market, so there is no edge”                | Checked our red bench against its own numbers                                                                              | Red’s own fair value was \$6–9 against a \$13.14 tape — a 35–55% disagreement <i>with the market</i>                                                                         | <b>NO — our own bench mislabeled its strongest arithmetic as a null result</b>                                                                             |
| Privosegtor is worth nothing                                                                   | Blue bench priced it                                                                                                       | Breakthrough Therapy + PRIME, no approved therapy in the indication; \$150–250M is defensible                                                                                | <b>NO — red both assigned it \$0 and claimed the full \$529M was buying the first drug. Those cannot both be true</b>                                      |
| Is PREDICT-1 large enough to win on the known effect?                                          | Back-computed the standard error from the parent trial’s published confidence interval and rescaled for the smaller sample | Minimum detectable effect $\approx 4.6$ vs the parent’s 4.3 point estimate                                                                                                   | <b>Inputs confirmed; the equal-variance step is an assumption we did not verify</b>                                                                        |
| “Topline around year-end 2026”                                                                 | Enrollment rate from the Q2 press release and<br><a href="#">NCT07548632</a>                                               | $\sim 4.5$ patients/week implies last-patient-in late December, topline Q1–Q2 2027                                                                                           | <b>Plausible, not confirmed — all sites are now active and</b>                                                                                             |

| Claim                                      | How checked                                                 | Found                                                                                                           | Confirmed?<br>enrollment often accelerates  |
|--------------------------------------------|-------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------|---------------------------------------------|
| Can we get out?                            | Average volume                                              | 220,185 shares/day $\approx$ \$2.9M/day; 1% of that is 2,200 shares                                             | <b>Yes</b>                                  |
| Live price                                 | Broker snapshot tool was permission-denied in this session  | Verified instead against the same-session close, \$13.14 on 2026-08-17, which matches the evidence pack exactly | <b>Price yes; tool access gap disclosed</b> |
| Why was the <i>Cornea</i> paper retracted? | Attempted to retrieve the retraction notice                 | Not retrievable                                                                                                 | <b>UNVERIFIED. We assert no misconduct</b>  |
| Next earnings 2026-11-05                   | Derived from a market-data feed, not a company announcement | No Oculis press release names it                                                                                | <b>UNCONFIRMED</b>                          |

## PROPOSED ENTRY BANDS

**No entry. Size zero today.** Both benches decline in both directions, so what follows are the conditions that reopen the name.

**Reopen to buy — price: - \$9.50** — first look. This is roughly 2.1 $\times$  cash and the level at which a ~45% chance of approval on a \$300M-peak drug is underpaid rather than fairly paid (blue's corrected table: \$300M peak  $\Rightarrow$  \$16.14/share on approval). - **\$9.00 or below** — full re-court. Approximately 2 $\times$  cash; red's own stated mind-changer. - **\$10.52** (the 52-week low) — early warning only, not a buy. It means the reopen zone is live.

**Reopen to buy — evidence, at any price:** - The RELIEF genotype subgroup's Day-29 *symptom* result disclosed with sample size and confidence interval — that would be the first independent replication on the endpoint that decides approval. - The *Cornea* retraction notice text, if it shows an authorship or administrative cause with the data intact. - A company statement of an FDA agreement that PREDICT-1 alone supports a marketing application, with a defined companion-diagnostic path. - PIONEER-1 interim data on privosegtor (NCT07623668).

**Do not buy a positive topline print.** Per the financing pattern above, wait for the equity offering to clear before pricing the win. If we ever own it, the reference trim levels from the corrected table are **\$16.14** (the price implied by a \$300M-peak approval) and **\$19.71** (+50% from here); the hard floor under any position is the **\$4.42–\$4.57/share** of cash.

**Sizing if a gate hits:** the ceiling is liquidity, not conviction. At 1% of average daily volume — 2,200 shares, about \$21,000/day at \$9.50 — a position we can exit inside ten sessions is roughly **22,000 shares, about \$210,000**. Anything larger is an unexitable position into a result that can print on any calendar day.

**Short:** refused. A cash floor at a third of the price, an undated binary that can be announced any morning, and 2,200 shares a day of liquidity is a squeeze waiting to happen.

**Catalyst dates:** - **2026-11-05** — Q3 results. Derived from a data feed, not confirmed by the company. - **2026-12-31** — registry primary completion date for PREDICT-1. Our own enrollment math says this slips. - **Q1–Q2 2027** — realistic PREDICT-1 topline, releasable by a 6-K on any day without warning. - **2027-02-28** — outside date for the Privosegtor acquisition closing from Accure. Stated in the company's own filing. - **2027-03-31** — outside date for the ACT-02 closing, which is conditional on the Privosegtor closing happening first. - Undated: PIONEER-1 interim; the full Accure agreement, which the company says it will file no later than with its annual report for fiscal 2026.

## ARMED SENSORS

---

(armed provisionally with this deck; session adjudication ratifies)

- price alert: fires below 10.52 (deck-provisional: 52-week low breached - advance warning that the reopen zone is live, not a buy)
- filing watch: 6-K until 2027-02-13 (deck-provisional: PREDICT-1 topline is releasable by a foreign-issuer report on any calendar day with no scheduled date )
- dated pack OCS|2026-11-05: OCS: Q3 2026 results - data-feed derived, no company release names it (date confirmed=False)
- dated pack OCS|2026-12-31: OCS: Registry primary completion for PREDICT-1; our enrollment math says this slips (date confirmed=False)
- dated pack OCS|2027-03-31: OCS: Realistic PREDICT-1 topline window (Q1-Q2 2027) per 4.5 patients/week enrollment arithmetic (date confirmed=False)
- dated pack OCS|2027-02-28: OCS: Outside date for the Privosegtor acquisition closing, per the 6-K of 2026-08-10 (date confirmed=True)

## What remains unverified

---

- **Why the *Cornea* paper was retracted.** The notice is not retrievable. We do not allege misconduct, and a purely administrative retraction would materially change the reading.
- **The exact share count.** We are working from loss-per-share arithmetic, not a primary count; secondary sources span 60.39M–61.73M. That is  $\pm 4$ M shares, about  $\pm \$50$ M of enterprise value.
- **The power calculation's variance assumption.** We assumed the same variance in the genotype subgroup as in the parent population. That is standard but unproven, and enriched subgroups sometimes have lower variance, which would help the trial.
- **The right sales multiple and discount horizon.** Our own two benches disagree by a factor of two on what peak sales the stock needs. We did not resolve it.
- **Whether the FDA has agreed that one trial plus a companion diagnostic supports a filing.** No public statement either way.
- **Whether European sales belong in the model at all.** The company is Swiss with an EMA path, but nothing is filed.
- **The 2026-11-05 print date.** Feed-derived, never confirmed by the company.

- **Live broker pricing.** The snapshot tool was unavailable this session; the price was confirmed against a same-session close instead.

**Primary sources:** [6-K, 2026-08-10, Accure asset purchase agreement — accession 0001193125-26-341298, quoted directly in the evidence pack · PMC9271287 — parent Phase 2, primary endpoint met, not retracted · PMC11365598 — the retracted subgroup paper · NCT07548632 — PREDICT-1 · NCT05896670 — RELIEF, signs-only primary](#)

```
{"price_gates": [{"level": 9.50, "direction": "below", "basis": "First-look  
reopen: ~2.1x cash, where a ~45% approval chance on a $300M-peak  
drug is underpaid rather than fairly paid"}, {"level": 9.00,  
"direction": "below", "basis": "Full re-court: ~2x cash per share,  
the red bench's own stated mind-changer"}, {"level": 10.52,  
"direction": "below", "basis": "52-week low breached – advance  
warning that the reopen zone is live, not a buy"}], "event_gates":  
[{"forms": ["6-K"], "why": "PREDICT-1 topline is releasable by a  
foreign-issuer report on any calendar day with no scheduled date –  
the undated binary"}, {"forms": ["424B5", "F-3", "F-3ASR", "S-8"],  
"why": "Live shelf plus demonstrated habit of selling into strength  
– a positive readout is likely a financing event that harvests the  
upside"}, {"forms": ["6-K", "20-F"], "why": "Accure closings  
(Privosegtor, then ACT-02) and the full agreement text due no later  
than the FY2026 annual report"}, {"forms": ["CTGOV-NCT07548632"],  
"why": "Registry status, last-patient-in and primary completion  
changes reveal the real timeline before any press release"},  
{"forms": ["CTGOV-NCT07623668"], "why": "PIONEER-1 privosegtor  
interim – the asset carrying $150-250M of the current price"}],  
"catalyst_dates": [{"date": "2026-11-05", "what": "Q3 2026 results –  
data-feed derived, no company release names it", "confirmed":  
false}, {"date": "2026-12-31", "what": "Registry primary completion  
for PREDICT-1; our enrollment math says this slips", "confirmed":  
false}, {"date": "2027-03-31", "what": "Realistic PREDICT-1 topline  
window (Q1-Q2 2027) per 4.5 patients/week enrollment arithmetic",  
"confirmed": false}, {"date": "2027-02-28", "what": "Outside date  
for the Privosegtor acquisition closing, per the 6-K of 2026-08-10",  
"confirmed": true}, {"date": "2027-03-31", "what": "Outside date for  
the ACT-02 closing, conditional on the Privosegtor closing, per the  
same 6-K", "confirmed": true}], "immediate_entry": null}
```